

**Providence Little Company Of Mary
Medical Center San Pedro**1300 W 7TH ST
SAN PEDRO CA 90732-3505
310-832-3311Admit Date: 7/7/2026 1204
Inp Admit Date/Time: 7/7/26 8:20 PM
Hospital Account: 772000817031
MRN: 20005877162
Patient: GONZALEZ, ERIC
Contact Serial #: 51072811111
Private Encounter? No**ENCOUNTER**Patient Class: Inpatient Unit: CLS TELEMETRY
Hospital Service: General Medicine Room/Bed: 372/372-01
Admitting Provider: Ali Fadhil, MD Referring Physician:
Attending Provider: Rudo Benjamin, MD Adm Diagnosis: Cellulitis of right groi*
Admit Source: Non-healthcare facility * Clinical Impression: Cellulitis of right groi*
Advanced Directives: No-Information Given Admit Type: Emergency**PATIENT**Name: GONZALEZ, ERIC DOB: 8/31/1973 (52 yrs)
Address: ~~661 N Harbor Blvd Apt 74~~ *Golden State Recuperative Care* Legal Sex: Male
~~SAN PEDRO, CA 90731~~ *153 E 110th St* Gender Identity:
Los Angeles, 90061 Patients Primary Phone: 424-450-5249
Primary Care Mark D Dressner PCP Phone: 562-933-0400**EMERGENCY CONTACT**

Contact Name	Legal Guardian?	Relationship to Patient	Home Phone	Primary Phone
1. Michel, Leonor		Friend	424-339-4452	424-339-4452
2. *No Contact Specified*				

GUARANTORGuarantor: Gonzalez, Eric DOB: 8/31/1973
Address: 661 N Harbor Blvd Apt 74 Sex: Male
SAN PEDRO, CA 90731 Home Phone: 424-450-5249
Relation to Patient: Self Work Phone:
Guarantor ID: 2424792**GUARANTOR EMPLOYER**

Employer: Status: NOT EMPLO*

COVERAGE**PRIMARY INSURANCE**Insurance Company: LA CARE HLTH PLAN MEDICAID HMO Plan: LA CARE MDCHD HMO HEALTHCARE
Group Number: MWLLNMEG21 Insurance Type: Indemnity [1]
Subscriber Name: GONZALEZ, ERIC Subscriber DOB: 08/31/1973
Pat. Rel. to Subscriber: Self Subscriber ID: 91440693A
Phone Number: (888)452-2273 HIC #:
Billing Flag:**SECONDARY INSURANCE**Insurance Company: Plan:
Group Number: Insurance Type:
Subscriber Name: Subscriber DOB:
Pat. Rel. to Subscriber: Subscriber ID:
Phone Number: HIC #:

Contact Serial # (51072811111)



Chart ID (20005877162-CLS ENCONTE-11)



7/11/26, 1:50 PM

Outpatient Referral Form

Ordering Site Information:	Physician Information:
Location: PROVIDENCE LCOM SAN PEDRO Address: TELEMETRY 1300 W 7TH ST SAN PEDRO CA 90732-3505 Phone: 310-514-5357 Fax: 310-514-5413	Enc. Provider: Rudo Benjamin, MD Auth. Provider: Rudo Benjamin Encounter Number: 5107281111 Electronically Signed.

Patient Information:

Name: Gonzalez, Eric
Legal Sex: Male
Gender Identity: Male
Date of Birth: 8/31/1973
Age: 52 year old

Patient ID: 20005877162
Pref Language: English
Phone: 424-450-5249 (home)
Address: 661 N Harbor Blvd Apt 74
 San Pedro CA 90731

Referral to Home Health - OUTPATIENT [2231699788]

Electronically signed by: Rudo Benjamin, MD on 07/10/26 1504

Status: Active

Ordering user: Rudo Benjamin, MD 07/10/26 1504

Ordering provider: Rudo Benjamin, MD

Ordering mode: Standard

Authorizing Provider Audit Trail

Date/Time	Authorizing Provider	Changed by
7/10/2026 3:04 PM	Rudo Benjamin, MD	Rudo Benjamin, MD

Ordering Provider Audit Trail

Date/Time	Ordering Provider	Changed by
7/10/2026 3:04 PM	Rudo Benjamin, MD	Rudo Benjamin, MD

Referral Information

Procedure

Referral to Home Health - OUTPATIENT [REF34]

Referral ID

Referred By

Referred To

92032489

BENJAMIN, RUDO

Not Available

Visits

Status

Start Date

End Date

1

Pending Review

7/10/26

7/10/27

Referral Details

Referred By

Referred To

Priority

Rudo Benjamin, MD
 4101 TORRANCE BLVD
 CHE BLDG, 2ND FLOOR
 TORRANCE CA 90503
 Phone: 310-303-6859
 Fax: 310-303-6834

Diagnoses: Cellulitis of right groin
 Pressure sore of hip, right, unstageable (HCC)
 S/P AKA (above knee amputation) unilateral, right (HCC)
Order: Referral To Home Health - Outpatient
Reason: Specialty Services Required

Routine

Associated Diagnoses

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	ICD-10-CM	ICD-9-CM
Cellulitis of right groin - Primary	L03.314	682.2
Pressure sore of hip, right, unstageable (HCC)	L89.210	707.04 707.25
S/P AKA (above knee amputation) unilateral, right (HCC)	Z89.611	V49.76

Questions

Face to face encounter date	7/10/2026
Primary diagnosis for home health	Wound cellulitis
Patient is homebound because of	Unhealed wounds See primary diagnosis for home health
Clinical findings that support homebound status	Needs assistance of another individual
Patient is homebound because he/she cannot leave home without the assistance of another individual and leaving home requires a considerable and taxing effort. Patient needs the assistance of another individual to leave home because	High fall risk due to gait instability and muscle weakness caused by: (Use Comment)
comment	ccc
Provider to follow patient	PCP
Provider to follow	NO, PHYSICIAN
Primary services needed	Skilled Nursing Physical Therapy
Skilled Nursing	Home safety eval and needs assessment Assess/instruct wound care/incision care Assess/instruct disease
Physical Therapy	Mobility & home safety/fall prevention/durable medical equipment needs & training/caregiver training
Interpreter needed	No

Active Insurance as of 7/7/2026

Primary Coverage

Payor	Plan	Insurance Group	Employer/Plan Group
LA CARE HLTH PLAN MEDICAID HMO	LA CARE MDCD HMO HEALTHCARE LA IPA	MWLLNMEG21	
Payor Plan Address	Payor Plan Phone Number	Payor Plan Fax Number	Effective Dates
PO BOX 811580	888-452-2273		5/1/2026 - None Entered
LOS ANGELES CA 90081-0010			

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Subscriber Name	Subscriber Birth Date	Member ID	
GONZALEZ,ERIC	8/31/1973	91440693A	
Guarantor Name (ID)	Guarantor Birth Date	Guarantor Address	Guarantor Type
GONZALEZ,ERIC (2424792)	8/31/1973	661 N Harbor Blvd Apt 74 SAN PEDRO CA 90731	Personal/Family

Process Instructions

I certify this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy, or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care, and will periodically review the plan. I further certify this patient had a face-to-face encounter that was performed by a physician or Medicare allowed non-physician practitioner that was related to the primary reason the patient requires home health services.

(For credentialed Nurse Practitioners or Physician Assistants in the state of California):

I attest that I am working within my scope of practice, Practice Agreement, and/or Standardized Procedures which includes the authority to Refer/order home health services, Approve, sign, approve, modify, add to, update, certify/recertify/oversee the Home Health plan of care/treatment, and that I have consulted with treating/supervising physician/surgeon.

Referral to Home Health - OUTPATIENT [2231699788]

Electronically signed by: Rudo Benjamin, MD on 07/10/26 1504

Status: Active

Ordering user: Rudo Benjamin, MD 07/10/26 1504

Ordering provider: Rudo Benjamin, MD

Authorized by: Rudo Benjamin, MD

Ordering mode: Standard

Frequency: 07/10/26 -

Diagnoses

Cellulitis of right groin [L03.314]

Pressure sore of hip, right, unstageable (HCC) [L89.210]

S/P AKA (above knee amputation) unilateral, right (HCC) [Z89.611]

Authorizing Provider Audit Trail

Date/Time	Authorizing Provider	Changed by
7/10/2026 3:04 PM	Rudo Benjamin, MD	Rudo Benjamin, MD

Ordering Provider Audit Trail

Date/Time	Ordering Provider	Changed by
7/10/2026 3:04 PM	Rudo Benjamin, MD	Rudo Benjamin, MD

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Gonzalez, Eric (MRN:20005877162) Printed 7/11/2026 1:51:28 PM

Gonzalez, Eric

MRN: 20005877162



Rudo Benjamin, MD
Physician
Internal Medicine

Discharge Summary  
Signed

Date of Service: 07/11/26 1152
Creation Time: 07/11/26 1152

Pt. Name/Age/DOB: Eric Gonzalez 52 y.o. 8/31/1973

Medical Record Number: 20005877162

Date of Admission: 7/7/2026

Date of Discharge: 7/11/2026 11:52 AM PDT

Attending Physician: Rudo Benjamin, MD

Primary Care Physician: Mark D. Dressner

PROVIDENCE HEALTH AND SERVICES

HOSPITAL MEDICINE DISCHARGE SUMMARY

Chief Complaint:

Right groin pain with drainage

Principal Diagnosis

Cellulitis of right groin associated with infected wound and pressure injury, without abscess or necrotizing infection [present on admission]

Hospital Problem List

Cellulitis of right groin associated with infected wound and pressure injury, without abscess or necrotizing infection [present on admission]
Unstageable pressure injury of right groin [present on admission]
History of right above-knee amputation with prosthesis-related wound complication [present on admission]
Human immunodeficiency virus (HIV) infection on antiretroviral therapy [present on admission]
Normocytic anemia [present on admission]
Hypocalcemia [present on admission]
Left anterior knee pustular skin lesions [present on admission]
Unspecified mononeuropathy of right lower limb [present on admission]
Homelessness with social determinants impacting care [present on admission]

Procedures Performed During This Hospitalization

No procedures performed during this hospitalization.

Diagnostics Performed During This Admission

CT abdomen and pelvis with IV contrast (7/7/2026): Findings consistent with cellulitis of the right groin/thigh with suspected soft tissue ulceration; no abscess or soft tissue gas identified.

Consultations Obtained During This Hospitalization

Gregory T. Ginsburg, General Surgery, 07/08/2026

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Reason for Admission to the Hospital / History of Present Illness

52-year-old male with history of right above-knee amputation and HIV on BIKTARVY presented with one week of progressive right groin pain, redness, swelling, and purulent drainage at the prosthesis contact site. Denied fever or vomiting. Exam notable for malodorous purulent wound with surrounding erythema and tenderness. Admitted for IV antibiotics and evaluation of complicated soft tissue infection.

Hospital Course

The patient presented with **right groin cellulitis with purulent wound** at the site of prosthesis friction. Initial evaluation included CT imaging which demonstrated **soft tissue edema and cellulitis without abscess or necrotizing infection**. He was hemodynamically stable on admission without systemic signs of sepsis. Empiric IV antibiotics with **cefTRIAxone and vancomycin** were initiated, along with IV fluids and analgesia. General Surgery (Dr. Gregory T. Ginsburg) evaluated the patient and **recommended no surgical debridement**.

During hospitalization, the patient showed **clinical improvement in erythema, drainage, and pain**. Wound care was performed daily with **normal saline cleansing, collagenase (SANTYL), saline-moistened gauze, and Mepilex dressing**. Antibiotics were continued intravenously and then transitioned to **oral linezolid (ZYVOX)** prior to discharge based on clinical response. No abscess developed, and there was no indication for operative intervention.

Additional issues addressed included **unstageable pressure injury of the groin**, managed conservatively with wound care. **Left knee pustular lesions** were treated with topical mupirocin. **Hypocalcemia** was identified and supplemented. His **HIV remained stable on BIKTARVY**, which was continued without interruption. Physical therapy was involved for **wheelchair training**, and prosthesis use was held to allow wound healing.

Social work coordinated discharge due to homelessness, and the patient was accepted to a recuperative care facility with arrangements for transportation and durable medical equipment including a wheelchair.

On the day of discharge, the patient was **afebrile, hemodynamically stable, tolerating oral medications, and clinically improved**, and was deemed safe for discharge with close outpatient follow-up and continued wound care.

Time Spent: 35 minutes required to complete the discharge summary and orders.

Problem-Based Discharge Summary

Cellulitis of right groin associated with infected wound and pressure injury, without abscess or necrotizing infection [present on admission]

Presented with purulent wound, erythema, and tenderness. CT imaging confirmed **cellulitis without abscess or gas**. Treated with **IV cefTRIAxone and vancomycin**, transitioned to **oral linezolid (ZYVOX) 600 mg every 12 hours** at discharge. No surgical intervention per General Surgery (Dr. Ginsburg). Clinical improvement noted. Discharged on oral antibiotics and wound care.

Unstageable pressure injury of right groin [present on admission]

Managed with daily wound care including **collagenase (SANTYL)** and dressing changes. No debridement required. Continue outpatient wound care.

History of right above-knee amputation with prosthesis-related wound complication [present on admission]

Wound attributed to prosthesis friction. Prosthesis use held. Wheelchair provided and training completed. Outpatient prosthesis reassessment recommended.

Human immunodeficiency virus (HIV) infection on antiretroviral therapy [present on admission]

Stable on **BIKTARVY**, continued without interruption. Outpatient follow-up with primary care and infectious disease recommended.

Normocytic anemia [present on admission]

Hemoglobin 11.7 g/dL, MCV 89.1 fL. Stable, no acute intervention required.

Hypocalcemia [present on admission]

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Calcium 8.4 mg/dL. Supplementation provided during hospitalization.

Left anterior knee pustular skin lesions [present on admission]

Treated with topical mupirocin (BACTROBAN) with improvement.

Unspecified mononeuropathy of right lower limb [present on admission]

Chronic condition, stable during hospitalization.

Homelessness with social determinants impacting care [present on admission]

Discharge coordinated to recuperative care facility. Transportation and medications arranged.

Tests Outstanding at Discharge Requiring Follow Up

Blood cultures – preliminary results pending; follow-up with primary care physician

Discharge Condition

Stable, improved

Vital Signs

Temp: 36.8 °C

HR: 76

BP: 95/60

RR: 16

SpO2: 96 %

PHYSICAL EXAM

General: Alert, cooperative, no distress

HEENT: Pupils equally round and reactive to light, oropharynx moist

Neck: Supple, no JVD

CV: Regular rate and rhythm, normal S1/S2

Lungs: Clear to auscultation bilaterally

Abd: Soft, non-tender, bowel sounds present

Ext: Right above-knee amputation; right groin wound with dressing clean, dry, intact, mild erythema without fluctuance

Neuro: Alert and oriented x3, moves all extremities

Skin: Pink, non stageable pressure injury backside

Psych: Calm, appropriate

Discharge Diet

Diet general

Discharge Medications

New Medications

- collagenase (SANTYL) ointment — Apply topically daily
- HYDROcodone-acetaminophen (NORCO) 5-325 mg — Take 1 tablet every 6 hours as needed
- linezolid (ZYVOX) 600 mg — Take 1 tablet every 12 hours
- mupirocin (BACTROBAN) 2% ointment — Apply topically 3 times daily

Continued Medications

- acetaminophen (TYLENOL) 500 mg — 1–2 tablets every 6 hours as needed
- amitriptyline (ELAVIL) 10 mg — nightly
- BIKTARVY 50-200-25 mg — daily

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- OLANzapine (zyPREXA) 5 mg — every evening
- UNCODED DME — right above-knee prosthesis referral

Stopped Medications

- Heparin (DVT prophylaxis) — inpatient only

Discharge Disposition

Recuperative care facility

Follow Up Appointments

Mark D. Dressner, MD
TCC, 455 E. Columbia St., #201
Long Beach, CA 90806
562-933-0400
Schedule appointment as soon as possible

Discharging Physician

Rudo Benjamin, MD

Portions of this clinical documentation were generated or assisted by artificial intelligence-based tools. All content has been reviewed, edited, and verified for accuracy and clinical appropriateness by Rudo Benjamin, MD, who remains solely responsible for the medical decision-making and final documentation.

MIPS Discharge Checklist v1.1

IA. Heart Failure with reduced EF < 40% (HFrEF) now or in the past year?
NO, the EF is NOT <40%

IB. Heart Failure with reduced EF < 40% (HFrEF) now or in the past year?
NO, the EF is NOT <40%

II. Acute/subacute ischemic stroke or TIA history?
NO h/o ischemic stroke/TIA

III. Advance Care Planning (ACP) > 65 years old?
NO - Patient <65 years old

IV. Proton Pump Inhibitor (PPI) evaluation > 50 years old
NO- not applicable, no PPI

V. Vancomycin with MRSA Cellulitis
YES assessed VH271- Vancomycin ordered with MRSA infection identified with risk for MRSA infection documented or Vancomycin NOT ordered/discontinued lenozolid ordered

VI. C. Diff HIGH risk
NO noted risk/concern for c.diff this encounter

VII. Medication reconciliation
Completed G8427: The provider team has reviewed a list of current prescription and OTC medications including names, dosages and route of admission using all immediate resources available on the date of the encounter to perform a medication reconciliation.

VIII. Follow up Care
YES NOT MET (GA 561)- Patient discharged with counseling to follow-up with provider(s) documented in discharge summary

Routing History

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Date/Time	From	To	Method
07/11/26 1157	Rudo Benjamin, MD	Mark D Dressner	Fax

7/11/26, 1:51 PM

Gonzalez, Eric (MRN:20005877162) Printed 7/11/2026 1:51:41 PM

Gonzalez, Eric

MRN: 20005877162

Jason G Miranda, RN

Registered Nurse

Wound/Ostomy

Consults

Signed



Date of Service: 07/08/26 1515

Creation Time: 07/08/26 1515

Consult Orders

Inpatient consult to Wound Ostomy Nurse [2229527981] ordered by Ali Fadhil, MD at 07/07/26 2258

WOUND CARE NURSE EVALUATION NOTES:

This is a 52 year old male with past medical history of HIV, right AKA with prosthesis, tobacco dependence, RLE mononeuropathy, homelessness, MVA, anemia. Patient can turn & reposition independently. Dr Ginsburg from general surgery on case. Upon assessment, patient noted with:

>>>Right gluteal linear friction blister, now scabbed

>>>Left hip blanchable linear erythema from friction

>>>Left anterior knee with 2 tiny intact pustules surrounded by erythema

>>>Right groin Unstageable Pressure Injury from prosthesis. Wound size is 12 x 1 x 2 cm. There is 25% loosely adherent yellow slough. Moderate serosanguinous drainage. Periwound slightly erythematous. Wound present on admission

RECOMMENDATIONS:

>>>For right groin pressure injury: Cleanse w/ NS, pat dry, apply Santyl ointment then pak w/ saline moistened gauze cover w/ Mepilex Border Flex daily

>>>Hold off on wearing the right AKA prosthesis until wound is resurfaced

>>>For left anterior knee pustules:
Apply Bactroban ointment tid

Discussed recommendations with RN Cynthia. Thank you for the consult. Will sign off now. Defer to Dr. Ginsburg. RN to provide wound care. Re consult as needed

Jason Miranda MSN-Ed, RN, CWON FACCWS
Wound Care Nurse

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Gonzalez, Eric

MRN: 20005877162

Sachie K Maeda-Chen, PT
Physical Therapist
Rehab Therapy

Plan of Care  
Signed

Date of Service: 07/09/26 1300
Creation Time: 07/09/26 1403

CLS PROVIDENCE LITTLE COMPANY OF MARY MEDICAL CENTER SAN PEDRO

Physical Therapy Initial Evaluation, Discharge (7/9/26)

Recommended discharge disposition: placement for care (pt is homeless)

Post discharge recommendation: no further PT

Equipment Recommendations: wheelchair

Summary: Pt received supine agreeable to PT eval, cleared by RN Jennifer to see, pt was independent w/ bed mobility, transfer to and from chair stand/squat pivot on L LE, pt returned to supine to sleep, PT recommends WC for home and CM/SW to follow up with placement as pt is homeless.

Patient History: 52-year-old male with history of right above-knee amputation and HIV on Biktarvy (reports undetectable viral load and CD4 count in the 400s) presents with one week of progressive right groin pain, redness, swelling, and purulent drainage at the site where his prosthesis contacts the groin crease. He denies fevers or vomiting. Patient is homeless with limited access to care. On presentation, he was hemodynamically stable without fever. Exam notable for malodorous, purulent wound with surrounding erythema and tenderness in the right groin region. He received IV fluids, cefTRIAXone, vancomycin, and analgesia in the ED. Surgical consultation (Dr. Ginsburg) requested for evaluation for possible incision and drainage. Admitted for IV antibiotics and further management of suspected complicated soft tissue infection.

Prior Functional Level: Per SW intake-Pt reports that he is currently homeless, has been homeless since he was released from jail on June 26, 2026. Pt has been residing in the community. Pt went to Harbor Interfaith yesterday to be connected with resources.

PT Barriers to Return to Prior Living Environment: none, patient safe to discharge

Planned Interventions: (None)

Recommended Frequency: one time visit;

Precautions

Precautions/Limitations: no known precautions/limitations

Left Upper Extremity Weight-Bearing: full weight-bearing

Right Upper Extremity Weight-Bearing: full weight-bearing

Left Lower Extremity Weight-Bearing: full weight-bearing

Right Lower Extremity Weight-Bearing: (AKA)

Bed Mobility

Additional Documentation: supine to/from sit

Assistive Device: none

Supine to Sit, Level of Independence: independent

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Sit to Supine, Level of Independence: independent

Transfers

Additional Documentation: bed to/from chair, sit to/from stand

Bed-Chair, Level of Independence: modified independent

Chair-Bed, Level of Independence: modified independent

Bed-Chair-Bed, Assistive Device: (chair)

Range of Motion

L LE ROM: WFL

R LE ROM: WFL

Strength

L LE Strength: at least 3/5

R LE Strength: at least 3/5

Balance

Sitting Balance: Static: normal balance

Sitting Balance: Dynamic: normal balance

Pain/Comfort/Sleep

Indicators Of Pain: no indicators of pain present

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Goals

PT Time Calculation
PT Start Time: 1300
PT Stop Time: 1320
PT Total Time: 20
Timed TX Code Minutes: 0

Electronically signed by: Sachie K. Maeda-Chen, PT, 7/9/2026 2:03 PM PDT

Cosigned by: Marinelle Camilon, DO at 07/09/26 1411

Revision History

Date/Time	User	Provider Type	Action
07/09/26 1411	Marinelle Camilon, DO	Physician	Cosign
07/09/26 1403	Sachie K Maeda-Chen, PT	Physical Therapist	Sign

7/11/26, 1:52 PM

Gonzalez, Eric (MRN:20005877162) Printed 7/11/2026 1:52:33 PM

Gonzalez, Eric

MRN: 20005877162

**Gregory T Ginsburg, MD**Physician
SurgeryConsults
Signed

Date of Service: 07/08/26 0736

Creation Time: 07/08/26 0736

Providence Little Company of Mary**Gregory T. Ginsburg, M.D., F.A.C.S.****Surgery Consultation****Pt. Name/Age/DOB:** Eric Gonzalez 52 y.o. 8/31/1973**Medical Record Number:** 20005877162**Case Serial Number:** 51072811111**Date of Admission:** 7/7/2026**Date of Consultation:** DATE/TIME: 7/8/2026 7:36 AM PDT**Referring Physician:** No Physician on file**History of Present Illness:** Information is gathered from patient

This patient is a 52 year old male who has a history of HIV and is s/p right AKA due to a MVA in the past. The patient wears a prosthesis and apparently the prosthesis is worn and has caused erosion to the skin around the thigh. The patient states that this has been happening for a while, but that recently it started to smell and cause increased pain. The patient denies other issues.

Past Medical History:

Past Medical History

Past Medical History:

Diagnosis

	Date
• Broken internal right knee prosthesis, subsequent encounter	07/07/2026
• Dorsalgia, unspecified	07/07/2026
• Hx of AKA (above knee amputation), right (HCC)	
• Imprisonment and other incarceration	07/07/2026
• Insomnia	07/07/2026
• Other stimulant dependence, uncomplicated (HCC)	07/07/2026
• Tobacco use	07/07/2026
• Unspecified mononeuropathy of right lower limb	07/07/2026
• Vitamin D deficiency	07/07/2026

Past Surgical History

Past Surgical History:

Procedure

• ABOVE KNEE AMPUTATION

Laterality

Date

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RIGHT**Allergies:**

Allergies

Allergies

Allergen

- Sulfamethoxazole-Trimethoprim

Reactions

Unknown

Home Medications:

Prescriptions Prior to Admission

Medications Prior to Admission

Medication	Sig	Dispense	Refill
• acetaminophen (TYLENOL) 500 mg tablet	Take 1-2 tablets by mouth every 6 hours as needed for Pain.	60 tablet	0
• amitriptyline (ELAVIL) 10 mg tablet	Take 1 tablet by mouth at bedtime.	30 tablet	0
• bicitgravir-emtricitabine-tenofovir alafenamide (BIKTARVY) 50-200-25 mg per tablet	Take 1 tablet by mouth Daily. Indications: HIV Disease	30 tablet	0
• OLANzapine (ZYPREXA) 5 mg tablet	Take 1 tablet by mouth every evening.	30 tablet	0
• UNCODED DME	Patient referral for right above the knee prosthesis for prior amputation.	1 each	0

Current Medications:

Current Medications

Current Facility-Administered Medications

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• acetaminophen (TYLENOL) tablet 650 mg Or	650 mg	Oral	Q4H PRN	Ali Fadhil, MD		
• acetaminophen (TYLENOL) 160 mg/5 mL liquid 650 mg Or	650 mg	Oral	Q4H PRN	Ali Fadhil, MD		
• acetaminophen (TYLENOL) suppository 650 mg	650 mg	Rectal	Q4H PRN	Ali Fadhil, MD		
• aluminum & magnesium hydroxide-simethicone (MAALOX PLUS)	30 mL	Oral	Q4H PRN	Ali Fadhil, MD		

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REGULAR
STRENGTH)
200-200-20
mg/5 mL
suspension 30
mL

- | | | | | | |
|--|---------------|-------------|-----------|----------------|-------------------------------------|
| • amitriptyline
(ELAVIL) tablet
10 mg | 10 mg | Oral | Nightly | Ali Fadhil, MD | 10 mg
at
07/07/
26
2227 |
| • bicitgravir-
emtricitabine-
tenofovir
alafenamide
(BIKTARVY) 50-
200-25 mg per
tablet 1 tablet | 1
tablet | Oral | Daily | Ali Fadhil, MD | |
| • bisacodyl
(DULCOLAX)
suppository 10
mg | 10 mg | Rectal | Daily PRN | Ali Fadhil, MD | |
| • calcium
carbonate
(TUMS)
chewable tablet
1,000 mg | 1,000
mg | Oral | Q4H PRN | Ali Fadhil, MD | |
| • ceTRIAXone
(ROCEPHIN) 2
g in sterile water
20 mL syringe
(100 mg/mL) | 2 g | Intravenous | Daily | Ali Fadhil, MD | |
| • HYDROcodone-
acetaminophen
(NORCO) 5-325
mg per tablet 1-
2 tablet | 1-2
tablet | Oral | Q4H PRN | Ali Fadhil, MD | |
| • OLANzapine
(zyPREXA)
tablet 5 mg | 5 mg | Oral | QPM | Ali Fadhil, MD | 5 mg
at
07/07/
26
2227 |
| • ondansetron
(ZOFTRAN ODT)
disintegrating
tablet 4 mg
Or | 4 mg | Oral | Q6H PRN | Ali Fadhil, MD | |
| • ondansetron
(ZOFTRAN)
injection 4 mg | 4 mg | Intravenous | Q6H PRN | Ali Fadhil, MD | |
| • polyethylene
glycol
(MIRALAX) | 17 g | Oral | Daily PRN | Ali Fadhil, MD | |

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powder 17 g

• sodium chloride 0.9% (NS) infusion		Intravenous	Continuou s	Ali Fadhil, MD	100 mL/hr at 07/07/26 2231	New Bag at 07/07/26 2231
• vancomycin 1 g in sodium chloride 0.9% 250 mL IVPB	1 g	Intravenous	Q12H	Ali Fadhil, MD	250 mL/hr at 07/08/26 0252	1 g at 07/08/26 0252
• vancomycin per pharmacy		Other	Pharmacy Consult	Ali Fadhil, MD		
• zolpidem (AMBIEN) tablet 5 mg	5 mg	Oral	Nightly PRN	Ali Fadhil, MD		5 mg at 07/07/26 2227

Family History:

Family History

No family history on file.

Social History:

Social History

Socioeconomic History

- Marital status: Single
- Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Every Day
- Current packs/day: 0.30
- Types: Cigarettes
- Smokeless tobacco: Not on file

Vaping Use

- Vaping status: Never Used

Substance and Sexual Activity

- Alcohol use: Yes
- Alcohol/week: 6.0 standard drinks of alcohol
- Types: 6 Cans of beer per week
- Drug use: Not Currently
- Sexual activity: Not on file

Other Topics

- Not on file

Social History Narrative

- Not on file

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Social Drivers of Health**Tobacco Use: High Risk (7/7/2026)****Patient History**

- Smoking Tobacco Use: Every Day
- Smokeless Tobacco Use: Unknown
- Passive Exposure: Not on file

Vaping: Low Risk (7/7/2026)**Vaping**

- Vaping Use Status: Never user

Alcohol Use: Not on file**Depression: Not on file****Stress: Not on file****Physical Activity: Not on file****Social Connections: Not on file****Intimate Partner Violence: Not on file****Violence/Safety: High Risk (7/7/2026)****Abuse Screen**

- We ask all patients, do you feel safe in your living/school environment?: Interviewer has concerns
- Patient shows signs of physical or sexual abuse, medical neglect, untreated STI's and or torture: Yes

Financial Resource Strain: High Risk (3/4/2025)**Overall Financial Resource Strain (CARDIA)**

- Difficulty of Paying Living Expenses: Hard

Food Insecurity: Not on file (7/7/2026)**Recent Concern: Food Insecurity - Food Insecurity Present (7/7/2026)****Hunger Vital Sign**

- Worried About Running Out of Food in the Last Year: Often true
- Ran Out of Food in the Last Year: Often true

Housing: High Risk (7/7/2026)**Housing**

- Living Situation: I do not have a steady place to live (I am temporarily staying with others, in a hotel, in a shelter, living outside on the street, on a beach, in a car, abandoned building, bus or train station or in a park)
- Lives With: alone
- Living Arrangements: homeless
- Problems: Patient refused

Utilities: Not on file (7/7/2026)**Transportation Needs: Not on file (7/7/2026)****Recent Concern: Transportation Needs - Unmet Transportation Needs (7/7/2026)****PRAPARE - Transportation**

- Lack of Transportation (Medical): Yes
- Lack of Transportation (Non-Medical): Yes

Health Literacy: Not on file**Review of Systems:**

See information in HPI, no other pertinent findings

Exam:**Most Recent Vital Signs:**

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Temp: 36.7 °C (98 °F)	BP: 100/59	Heart Rate: 74	Resp: 18	SpO2: 98 % on
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Min/Max Temp past 24 hours: Temp Avg: 36.8 °C (98.3 °F) Min: 36.7 °C (98 °F) Max: 36.9 °C (98.5 °F)

I/O last 3 completed shifts:

In: -

Out: 400 [Urine:400]

No intake/output data recorded.

Weight most recent: Weight: 84.7 kg (186 lb 11.7 oz) **BMI:** Body mass index is 28.39 kg/m².

Physical Examination:

General: awake and alert

Cardiovascular: RRR

Respiratory: normal effort

Abdomen: soft

Genitourinary: n/a

Extremities: right above knee amputation stump is well-healed. There is a linear wound on the proximal anterior right thigh that extends down into the subcutaneous level and has some minimal necrosis noted at the base. I probed and spread the wound to get a good look and did not note deep purulence/fluctuance/tunneling to indicated deeper or more extensive involvement

Diagnostic Studies: Available Labs and Images were reviewed personally. Significant results and findings are addressed below or in the Assessment and Plan.

Recent Results (from the past 360 hours)

CT Abdomen Pelvis w IV Contrast

Narrative

**EXAM: CT ABDOMEN PELVIS W IV
CONTRAST**

EXAM DATE: 7/7/2026 10:06 PM.

CLINICAL HISTORY: Concern for Fournier gangrene. 52-year-old male with history of right above-the-knee amputation and HIV, one week of progressive right groin pain with redness, swelling, and purulent drainage at the site where his prosthesis contacts the groin crease. Exam revealed a malodorous purulent wound with surrounding erythema and tenderness.

COMPARISONS: None.

TECHNIQUE: Helical CT images of the abdomen and pelvis were obtained. IV contrast: IOHEXOL (OMNIPAQUE 350) 350 MG/ML INJECTION 100 ML. Enteric

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contrast: None. Reconstructions: Coronal and sagittal.

In accordance with CT protocol optimization, one or more of the following dose reduction techniques were utilized for this exam: automated exposure control, adjustment of mA and/or KV based on patient size, or use of iterative reconstructive technique.

FINDINGS:

Lung bases: Lung bases are clear. No pleural effusion.

Liver: Normal in size and contour. No focal lesion.

Gallbladder/Bile Ducts: Unremarkable.

Pancreas: Normal.

Spleen: Normal.

Adrenal Glands: Normal.

Kidneys/Ureters: Normal. No masses or hydronephrosis.

Bowel: The stomach and small bowel are normal. The colon is normal. The appendix is normal.

Pelvic Organs: The prostate and seminal vesicles are unremarkable.

Bladder: Normal.

Lymph Nodes: There is no mesenteric, retroperitoneal, or pelvic adenopathy.

Peritoneum/Retroperitoneum: There is no ascites or focal abnormality.

Vasculature: No aneurysm or other significant abnormality.

Bones: No significant abnormality.

Other: Soft tissue edema and skin thickening is seen in the anteromedial right thigh extending along the crease of the groin into the more anterior flank. Findings compatible with cellulitis. Some ulceration is

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suspected. No soft tissue gas is seen. No evidence of abscess.

Impression

IMPRESSION:

Findings suspicious for cellulitis and possible soft tissue ulceration in the area of concern. No evidence of soft tissue gas or other clear sign of necrotizing infection. Note that necrotizing infection cannot be excluded on imaging. Otherwise unremarkable exam.

RADIA

Radiation Dose Information Provided By Hospital/Clinic

*CT ABDOMEN PELVIS W IV CONTRAST
CTDI: 10.00 mGy CTDLP: 611.00 mGy-cm*

*Dictated By: Hurley, Patrick MD 07/08/2026
05:57 PDT*

*Signed By: Hurley, Patrick MD 07/08/2026
06:09 PDT*

*Transcribed By: Marchetti, Michelle
07/08/2026 06:00 PDT*

SITE ID: 355

Patients are advised to discuss imaging findings and recommendations with the ordering provider.

Assessment and Plan:

Problem List

Patient Active Problem List

Diagnosis

- Hx of AKA (above knee amputation) (HCC)
- Homeless
- HIV (human immunodeficiency virus infection) (HCC)
- Pneumonia
- Cellulitis of right groin
- Infected wound of right groin with cellulitis and visible subcutaneous tissue
- Dependence on wheelchair
- Harmful pattern of use of methamphetamine (HCC)
- Housing instability, housed unspecified
- Hyperlipidemia
- Unspecified mononeuropathy of right lower limb

Right thigh wound created by prosthesis

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I advised the patient to have the prosthesis evaluated as it probably needs either modification or replacement if he is to wear it again. Meanwhile, the wound does not appear to require surgical debridement. After cleansing the wound, we can apply topical Santyl to the wound daily for the small amount of necrotic tissue present. Since he also has HIV, the wound healing may be somewhat delayed. He should hold off on wearing the prosthesis until the wound is healed and the prosthesis gets repaired.

Electronically signed by: Gregory T. Ginsburg, M.D., F.A.C.S. 7/8/2026 7:36 AM PDT
CLS PROVIDENCE LITTLE COMPANY OF MARY MEDICAL CENTER SAN PEDRO